

Medical Record Access Policy

Policy owner: Health Information Management with Legal & Compliance oversight

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1. Purpose

This policy defines how authorized personnel may request, review, disclose and track access to patient medical records. The objective is to support continuity of care and legitimate operational needs while protecting confidentiality.

2. Scope and Definitions

Applies to clinical, patient-services, health-information, legal, billing and technology teams handling medical records. A medical record includes clinical notes, diagnostic reports, orders, results, medication records, images and related documentation maintained by MedCore.

3. Role-Based Access Model

Role	Typical access	Control
Treating clinician	Records required for active care	Role + care relationship
Nursing / care team	Records required for assigned care	Role + assignment
Health Information Management	Record administration and approved release	Role + workflow authorization
Billing / Finance	Minimum financial and coding information	Role + business need
Legal / Compliance	Records required for approved case or regulatory work	Case-specific authorization
IT support	Technical troubleshooting only	Break-glass / audited access where applicable

4. Access Request and Approval

Access outside a user's standard role must be requested through the approved access-management process. The request must identify the user, purpose, required systems or records, duration and approving authority.

Temporary access should have an explicit expiry. Managers are responsible for promptly requesting removal when a user's role changes.

5. Patient or Authorized Representative Requests

Requests for copies or access from patients or authorized representatives must be verified and processed through the designated Health Information Management workflow.

Identity verification, authority to act, applicable fees or exemptions, and any required redactions must be assessed before release.

6. Internal Record Review

Personnel may open a record only when there is a current care, operational, legal, compliance, quality or other approved business purpose.

Curiosity access, searching for relatives or colleagues without authorization, and accessing records after the business need ends are prohibited.

7. Disclosure Controls

Disclosure type	Required control
Patient / representative	Identity and authority verification; approved release workflow.
Care provider	Confirm legitimate care purpose and secure transfer route.
Insurer / payer	Use approved claims or authorization process; disclose minimum necessary data.
Regulator / law enforcement	Route to Legal/Compliance when authority or scope is unclear.
Internal investigation	Case-specific authorization and audit trail.

8. Emergency / Break-Glass Access

Emergency access may be used only when delay would create a material care or safety risk and normal access cannot be obtained in time. The user must select the approved emergency-access mechanism and provide the required reason.

Break-glass events are logged and subject to post-event review. Misuse may result in investigation and corrective action.

9. Audit and Monitoring

System access logs are reviewed according to risk and operational requirements. Reviews may examine unusual access patterns, high-volume record views, access outside assignment and break-glass events.

Suspected inappropriate access must be escalated through the incident process and should not be investigated informally by the individual who discovers it.

10. Retention, Release and Review

Released copies and related authorization records must be maintained according to the applicable retention schedule. The policy is reviewed annually or when significant regulatory, system or workflow changes occur.

Frequently Asked Questions

Can a doctor access any MedCore patient record?

No. Access is tied to role and legitimate purpose. A treating clinician generally needs records connected to active care, while other access requires an approved business need.

What if normal access is unavailable during an emergency?

Use the approved emergency or break-glass mechanism only when necessary, provide the required reason, and expect the event to be reviewed.

Can billing staff open the full clinical record?

Only where the approved role and business need require it. Billing access should be limited to information needed for the financial or coding task.